

IS THIS RIGHT FOR YOU?

When bariatric surgery may be appropriate.

Bariatric surgery may be considered when excess weight is contributing to long-term health risk and previous attempts at weight reduction have not produced durable results.

Type 2 diabetes

Blood sugar not well controlled despite multiple medications, or HbA1c persistently above 7.5%.

Elevated metabolic and cardiovascular risk

BMI 35 or above with one or more metabolic conditions. BMI 30 to 35 with severe insulin resistance or cardiovascular risk. BMI alone is not the determinant.

GLP-1 plateau or cessation

GLP-1 pharmacotherapy has plateaued, is unsustainable long-term, or has been ceased with significant weight recurrence.

Weight not responding to sustained effort

Two or more years of sustained lifestyle effort without durable metabolic benefit.

Other conditions associated with excess weight

High blood pressure, sleep apnoea, fatty liver disease, reflux, or joint pain affecting daily life.

COMING TO A CONSULTATION IS NOT A COMMITMENT

The assessment determines candidacy. No surgical decision is made before the full team has reviewed your case. Many people find the assessment itself clarifying.

GETTING STARTED

THREE WAYS TO TAKE THE FIRST STEP.

TALK TO YOUR GP

Ask your GP for a referral to Meridian Health.

Medicare rebates may apply with a valid GP referral.

CALL US DIRECTLY

(03) 9416 4418

Monday to Friday, 8:30am to 5:00pm

VISIT OUR WEBSITE

meridianhealth.com.au

Submit an enquiry online at any time.

WHERE WE ARE

Brenan Place

Aitkenhead Medical Discovery Precinct

Fitzroy, Melbourne

reception@meridianhealth.com.au

Opening September 2026

MERIDIAN HEALTH

MERIDIAN HEALTH

Bariatric Metabolic Surgery.

Personalised surgical options. Structured preparation. Twelve months of included follow-up.

Surgeon-led specialist care
Brenan Place, Fitzroy, Melbourne
(03) 9416 4418



Tailored procedures. Choosing yours is anything but routine.

Procedure selection is individualised around metabolic health, eating patterns, anatomy, and long-term treatment goals.

Laparoscopic sleeve gastrectomy

The most commonly performed bariatric procedure. A significant portion of the stomach is removed to reduce hunger and support durable weight reduction.

Roux-en-Y gastric bypass

Combines restriction and selective malabsorption. Often considered in patients with significant reflux, type 2 diabetes, or failed previous interventions.

One anastomosis gastric bypass

A single-anastomosis bypass with substantial metabolic and weight-loss effects in selected patients.

Sleeve fundoplication

Combines sleeve gastrectomy with anti-reflux surgery. Considered where obesity and reflux coexist.

Revision bariatric surgery

Assessment and management of previous bariatric procedures, including weight regain, nutritional concerns, or anatomical problems.

THE EVIDENCE, BRIEFLY

T2DM remission rates exceed those of any pharmacological intervention in selected surgical patients.

Long-term weight loss maintenance is superior to every non-surgical intervention at five and ten years.

From first consultation to long-term follow-up.

The first twelve months are considered part of the procedure itself.

01 Surgical consultation

A bariatric surgeon reviews medical history, weight trajectory, metabolic health and treatment goals. Available surgical pathways are discussed.

02 Assessment and investigations

Dietitian review, blood tests, body composition analysis. Additional investigations arranged where appropriate.

03 Pre-operative preparation

Two-week VLCD liver reduction diet, nurse education, pre-operative review and anaesthetic assessment.

04 Surgery and hospital stay

Laparoscopic procedure within connected private hospitals. Most patients stay one to two nights.

05 Twelve months of included follow-up

Structured review with the surgical, nursing and dietetic team throughout the first year. Included within the surgical fee.

06 Long-term metabolic care

Many patients continue long-term follow-up. Ongoing review supports durable weight maintenance and nutritional health.



SURGEON
A/Prof Michael Hii
Oesophago-Gastric & Bariatric Surgeon
FRACS · MBBS · BMedSci · PGDipSurgAnat



SURGEON
Dr Salena Ward
Gastric, Bariatric & General Surgeon
MBBS · FRACS · MS · PGDipSurgAnat



SURGEON
Dr Nicole Winter
Oesophago-Gastric & Bariatric Surgeon
FRACS · MBBS (Hons) · BMedSci · AANZGOSA

INDICATIVE TWELVE-MONTH OUTCOMES

T2DM patient, post bariatric metabolic surgery

HbA1c 8.4% to 5.9% Normal range	Medications 4 agents to 2 Continuing to taper
Blood Pressure 138/88 to 118/76 Normalised	Triglycerides 3.2 to 1.1 mmol/L Normal range

Representative outcomes. Individual results vary. Published with patient consent.